Health system at central, state and local levels

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Health

- According to WHO, Health is a state of complete physical, mental and social well being and not merely an absence of disease or infirmity.
- Health is an ability to lead a socially and economically productive life.

Health care

Health care is the diagnosis, treatment and prevention of disease illness, injury and other physical and mental impairments in human beings.

Health care defined as a ‘multitude of services rendered to individuals, families or communities by the agents of the health services or profession

Health care delivery system

The prevention, treatment and management of illness and preservation of mental and physical wellbeing through the services offered by the medical and allied health professions.

WHO defines it as an integrated care containing promotive, preventive and culture elements that bear the longitudinal association with an individual, extending from womb to tomb, and containing in the state of health as well as disease.

Determinants

- **Consumers of health care-** the public to whom heath care services are rendered.
- **Providers of health care-** Authorized health man power responsible to provide health care services to people.
- **Funding Sources-** Functioning Sources can be govt. sources, private sources, voluntary contribution from people, social and private insurance through joint contribution.
- **Others**- they include constitutional obligations, political system, ideology, agenda, health policies, judiciary obligation & control, executive machinery etc.

Health System in India

STRUCTURE

- India is a union of 29 states & 7 union territories.
- Under the constitution of India the states are largely independent in matters relating to the delivery of health care to the people.
- The central responsibility consists mainly of policy making, planning, guiding, assisting, evaluating & coordinating the work of sate health ministers

The Health system in India has 3 main links

- I. Central level
- II. State level
- III. Peripheral or district level

AT THE CENTRAL LEVEL

- The ministry of health and family welfare
- The directorate General of Health Services
- The Central Council of Health and Family welfare

The ministry of health and family welfare (MOHFW)

- **Organization**:

The union ministry of H & FW is headed by

- a Cabinet Minister,
- a Minister of State & a Deputy health Minister

Currently the ministry of H & FW is divided into:

- ✓ Department of Health
- ✓ Department of Family welfare.

Cont..

- The health Dept. is headed by secretary to GOI assisted by Joint secretaries, deputy secretaries & administrative staff.
- The Dept. of FW was created in 1966. The secretary to GOI in Ministry of H & FW is in overall charge of Dept. of FW. He is assisted by an additional secretary & commissioner & one joint secretary.

Functions

- The functions of the Union health ministry under

a). Union list

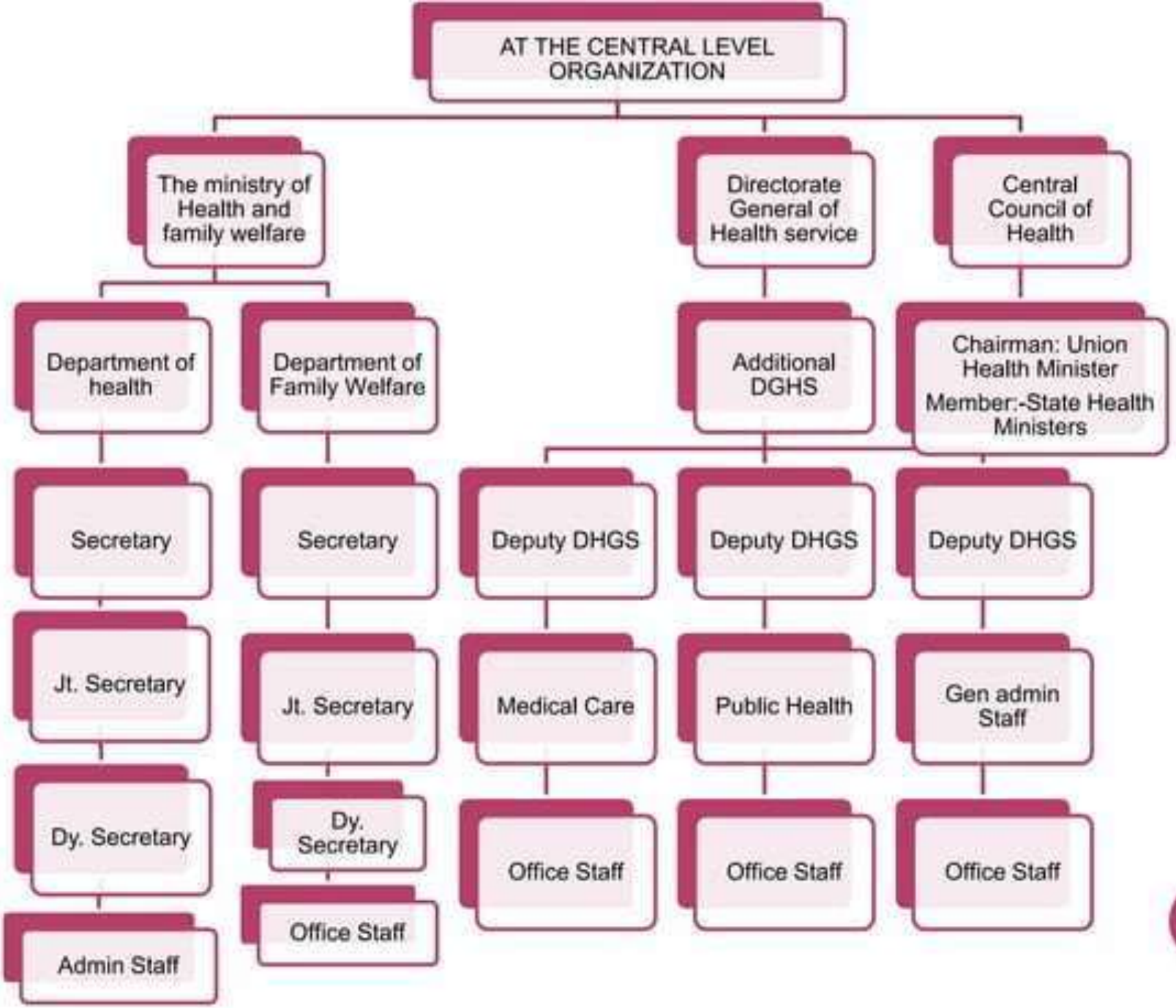
b). Concurrent list

Union list

- International health relation and administration of port quarantine.
- Administration of central institutes such as the All India institute of Hygiene and public health, Kolkata national Institute for the control of communicable disease Delhi etc.
- Promotion of Research through Research bodies.
- Regulation and development of medical, pharmaceutical, dental and nursing professions.
- Establishment and maintenance of drug standards.
- Census, collection and publication of other statistical data.
- Immigration and emigration.
- Regulation of labour in the working of mines.
- Co ordination with state and other ministries for promotion of health.

Concurrent list

- Prevention of extension of communicable disease from one unit to another.
- Prevention of adulteration of food stuffs.
- Control of drugs and poisons.
- Vital statistics.
- Labour welfare.
- Ports other than major.
- Economic and social planning.
- Population control and family planning.


AT THE CENTRAL LEVEL ORGANIZATION
The ministry of Health and family welfare
Directorate General of Health service
Central Council of Health
Department of health
Department of Family Welfare
Additional DGHS
Chairman: Union Health Minister
Member:-State Health Ministers
Secretary
Secretary
Deputy DHGS
Deputy DHGS
Deputy DHGS
Jt. Secretary
Jt. Secretary
Medical Care
Public Health
Gen admin Staff
Dy. Secretary
Dy. Secretary
Office Staff
Office Staff
Office Staff
Office Staff
Admin Staff

Directorate General of health services (DGHS).

- **Organization:**

The DGHS is the principal adviser to the Union Govt. in both Medical & Public health matters. He is assisted by an additional Director General of health services a team of deputies & administrative staff.

It comprises of 3 units :

- ✓ Medical care,
- ✓ Public health
- ✓ General administration

Functions

- General
- Specific

General

- Surveys
- Planning
- Coordination
- Programming
- Appraisal of all health matters.

Specific Functions

- **International Health relation and quarantine.**

All the major posts in the country and international air ports are directly controlled by the Directorate general of Health services

- **Control of drug standards**

  The drugs control organization is a part of duties and is controlled by the drug controller. Its primary function is to lay down the standards and control of the manufacture and distribution of the drugs through both central and state govt. offices.

- **Medical store depots**

The Union government runs medical store depots at Mumbai, Chennai, Kolkatta, Karnal, Gaunahati and Hydrabad. The medical store organization endeavors to ensure the highest quality, cheapest bargain ad prompt supplies.

- **Post graduate training**

The DGHS responsible for the administration of national institutes, which also provide post graduate training to different categories of health personal.

- **Medical education**

The DGHS directly charges in the medical colleges in India

CONT..

- **Medical Research**

  Medical research in the country is largely through the Indian council of medical research. The Council plays a significant role in aiding promoting and coordinating scientific research on human research on human diseases, their caustic presentation and cure.
- **Central Govt. Health scheme**
- **National Health programmes**

  The various national health programmes for the eradication of malaria and for the control of TB, filarial, leprosy, AIDS and other communicable disease involve expenditure of crores of rupees.
- **Central health education bureau.**

  An outstanding activity of the Bureau is preparation of education material for creating health awareness among the people. The Bureau often training causes health education to different categories of health workers.
- **Health intelligence**

  The CHEB was established in 196I to contribute collection, completion, analysis, evaluation and dissemination of all information on health statistics for the nation as a whole.
- **National Medical library**

  The central library of the DGHS was declared as National medical library in 1966.

3. The central council of Health (CCH).

The CCH was set up by a presidential order on 9/8/1952 under article 263 of constitution of India. The Union Health Minister is the chairman & the state health ministers are the members

Functions

- To consider and recommend broad outlines of policy in regard to matters concerning health in all its aspects such as the provision of remedial and preventive care, environmental hygiene, nutrition, health education and the promotion of facilities for training and research.
- To make proposal for legislation in fields of medical and public health matters and to lay down the pattern of development of the country as a whole.
- To make recommendation to the central government regarding distribution of available grants -in-aid for health purpose to the state and review periodically the work accomplished in different areas through the utilization of these grants in aid.
- To establish any organization or organizations invested with appropriate functions for promoting and maintaining cooperation between the central and state health administration.

II AT THE STATE LEVEL

At present there are 29 states in India, with each state having its own health administration.

The management sector comprises of :

1. State Ministry of health
2. State health directorate

CONT..

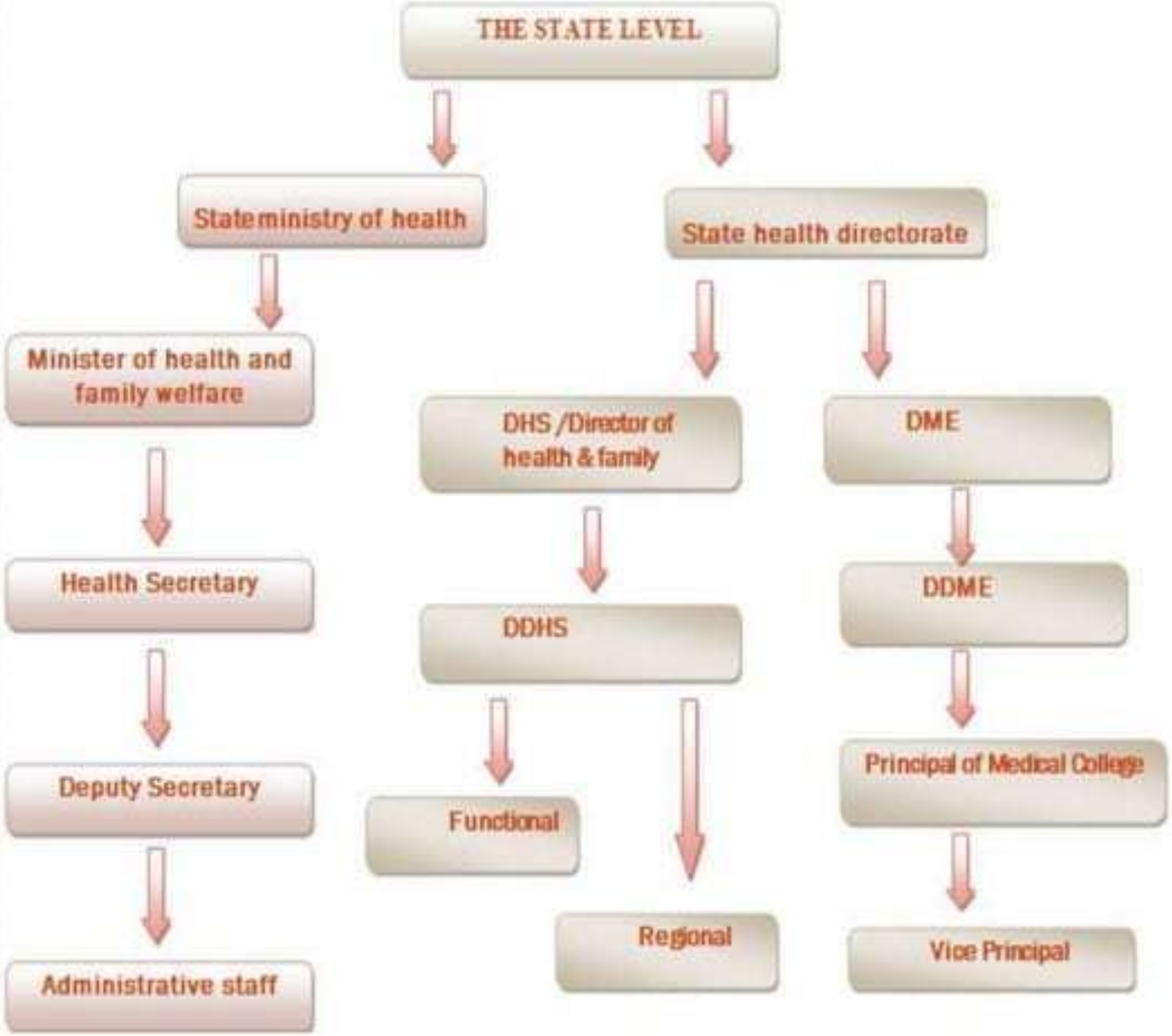
- **State ministry of health**
  - headed by a Minister of H & FW & Deputy Minister of H & FW.
  - The health secretariat is the official organ of state Ministry of health and is headed by a Secretary and assisted by deputy secretaries and administrative staff.

CONT..

- **State health directorate**
- The directorate of health services is the chief technical adviser to the state govt. on all matters relating to medicine and public health.
- With the advent of FP as an important programme, the designation of DHS has been changed as Director of health & FW. They are assisted by deputies & assistants.
- The deputy & assistant directors of health may be of 2 types – Regional & functional.
- Regional directors inspect all the branches of public health within their jurisdiction.
- Functional directors are specialist in a particular branch of public health.
- Recently there is an appointment of DME in view of increasing no. of medical colleges.

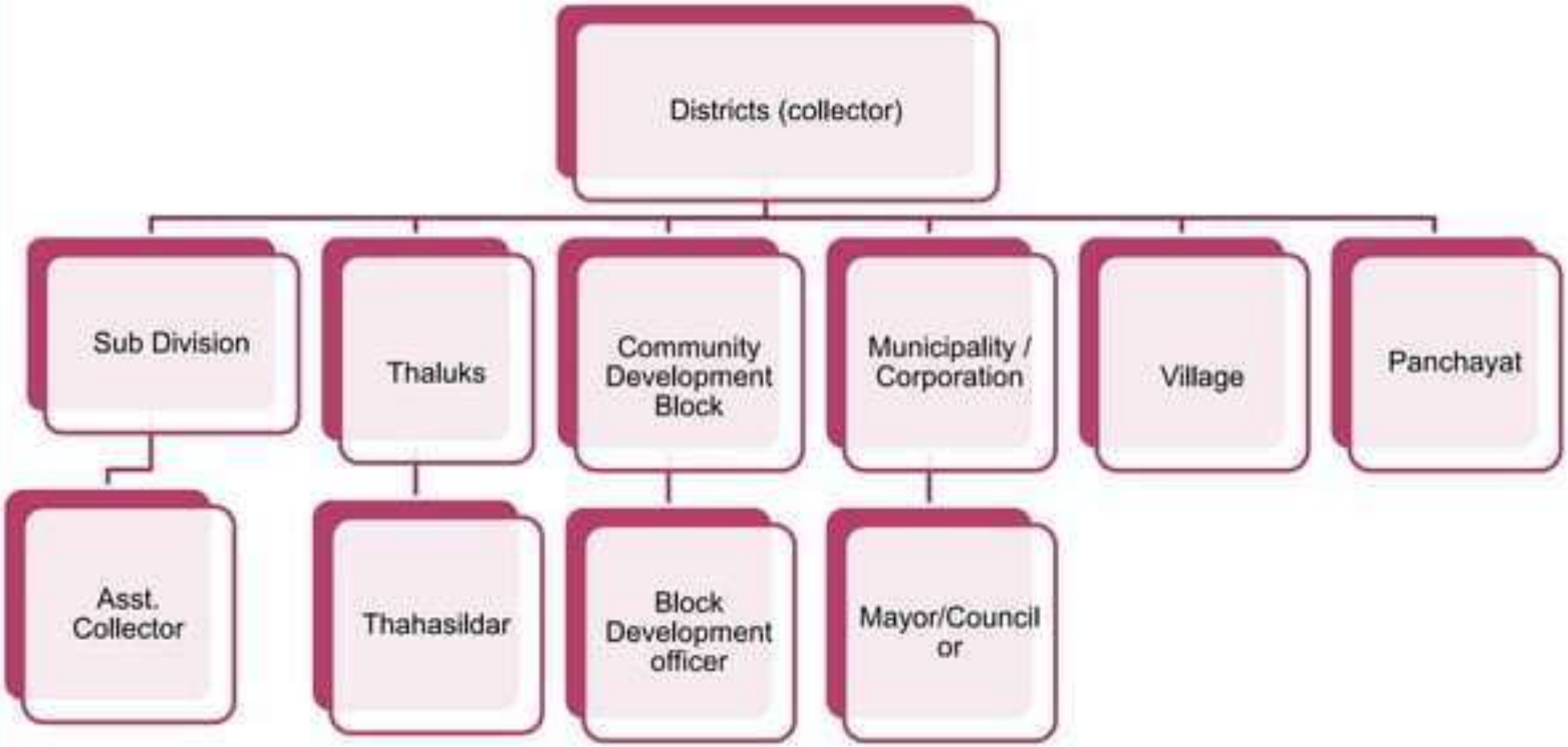
Functions

- Technical advice to state government on matters related to medicine and public health.
- Organization or direction of all health activities.
- Assisting in planning for health services in state.
- Implementation of national health programmes and evaluating their achievements.
- Providing all types of health services in the state.
- Controlling food adulteration and sanitation.
- Collection of vital statistics.
- Encouraging reproductive and child health.
- Improvement of nutritional programmes and medical education.
- Training of nurses, female health workers and other health workers.
- Controlling rural and urban health services through district medical officer.


THE STATE LEVEL
Stateministry of health
State health directorate
Minister of health and family welfare
Health Secretary
Deputy Secretary
Administrative staff
DHS /Director of health & family
DDHS
Functional
Regional
DME
DDME
Principal of Medical College
Vice Principal

III. AT THE DISTRICT LEVEL

- The principle unit of administration in India.
- There are 614 districts in India.
- Districts are known as ‘Zila’
- With each district again, there are 6 types of administrative areas.


Districts (collector)
Sub Division
Thaluks
Community Development Block
Municipality / Corporation
Village
Panchayat
Asst. Collector
Thahasildar
Block Development officer
Mayor/Council or

- Most districts India are divided into two or more subdivisions each in charge of an assistant collector or sub collector.
- Each division is again divided into tehsils, and tehsildar. And tehsildar is the in charge of one tehsil usually consists of 200 to 600 villages.
- The block is a unit of rural planning and development, and comprises approximately 100 villages, and about 80,000 to 1,20,000 population in charge of a block development officer.

- Town area committee - in areas with population ranging from 5000 and 10000.
- Municipal boards - In areas with population ranging between 10,000 and 2 lakhs.
- Corporation - with population above 2 lakhs.
- Town area committees are like panchayaths. They provide sanitary services.
- The municipal board is headed by a chairman/president, elected usually by the members.

- Functions
- Construction and maintenance of roads
- Sanitation and drainage.
- Street lighting
- Water supply
- Maintenance of hospitals and dispensaries.
- Education
- Registration of birth & death

- The administration at district level is being described under 2 headings:

- Urban administration
- Rural administration(Panchayati raj)

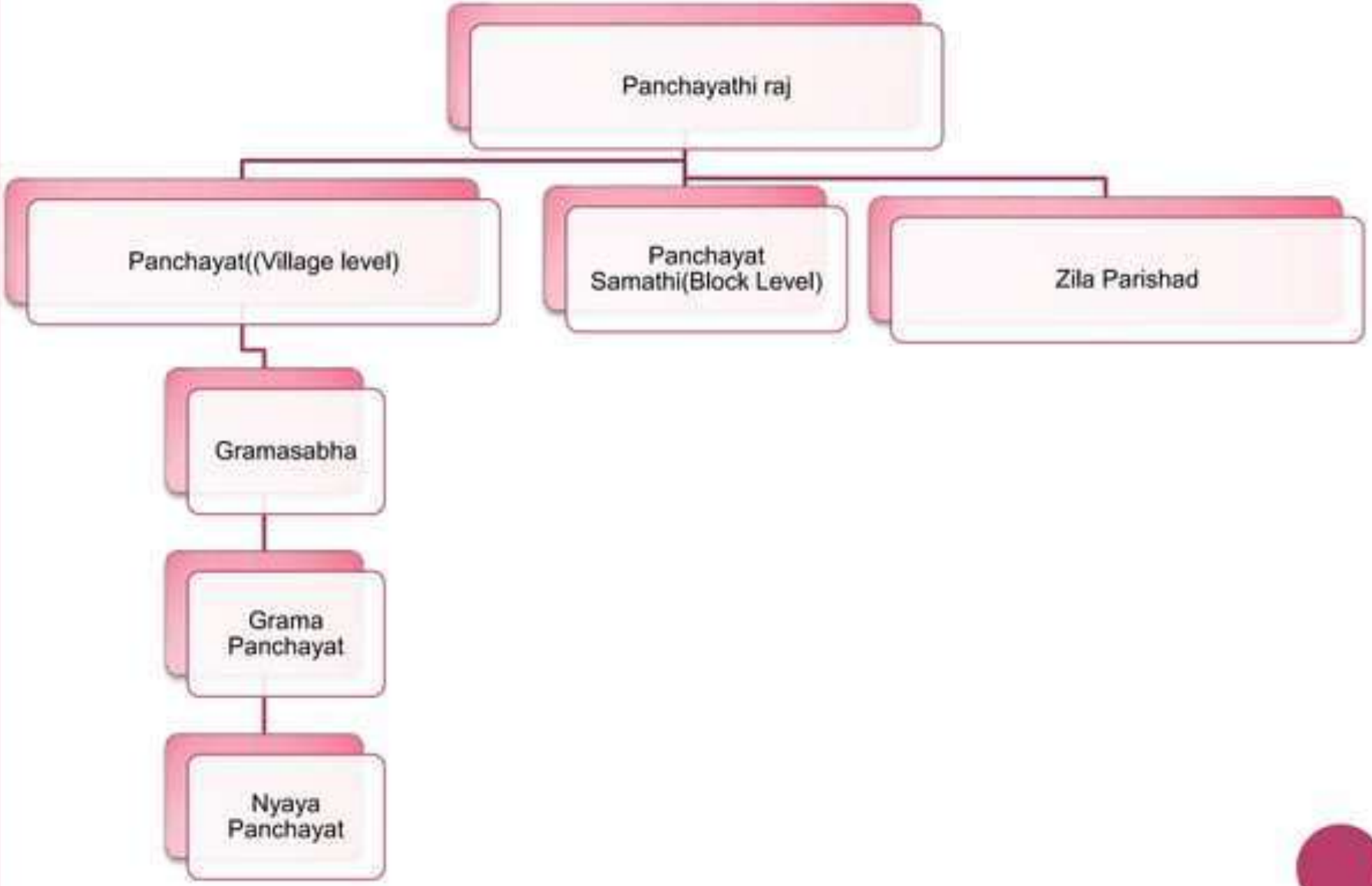
Urban Administration

The main units of urban administration are:

- **Municipal Corporation:** It can be said as a top level urban local government. It is constituted where the population of city is above 2 lakhs.
- **Municipality (Municipal boards):** It is constituted where the population of city or town ranging between 10,000 to 2 lakhs. It has three components: Chairman, board and municipal commissioner or executive officer. Municipality is headed by the chairman who is elected by board members.
- **Town area committees:** commonly towns are such areas, which are in between village and city. Town area committee is under the administration of district collector. Town area committees cover the population ranging between 5,000 to 10,000 and responsible for sanitation in the area.

Rural Administration (Panchayathi Raj)

- Panchayathi raj is a 3 tier structure of rural local self government.
- Linking the village to the district.


Panchayathi raj
Panchayat((Village level)
Panchayat Samathi(Block Level)
Zila Parishad
Gramasabha
Grama Panchayat
Nyaya Panchayat

At the block level

- Is known as Panchayat samathi
- Members are:
  - All sarpanches of the village panchayat in the Block,
  - MLAs, MPs residing in the block area,
  - representatives of women and scheduled tribes and cooperative societies.
- The main function is community development programme.

Panchayath at the Village level

- Gram sabha
- Gram Panchayath
- The Nyaya panchayath

CONT..

Gram sabha

- It is comprised of all the adult man and woman in the village.
- This body meets at least twice in a year and discusses important issues. They elect members of panchayat.

Gram Panchayath

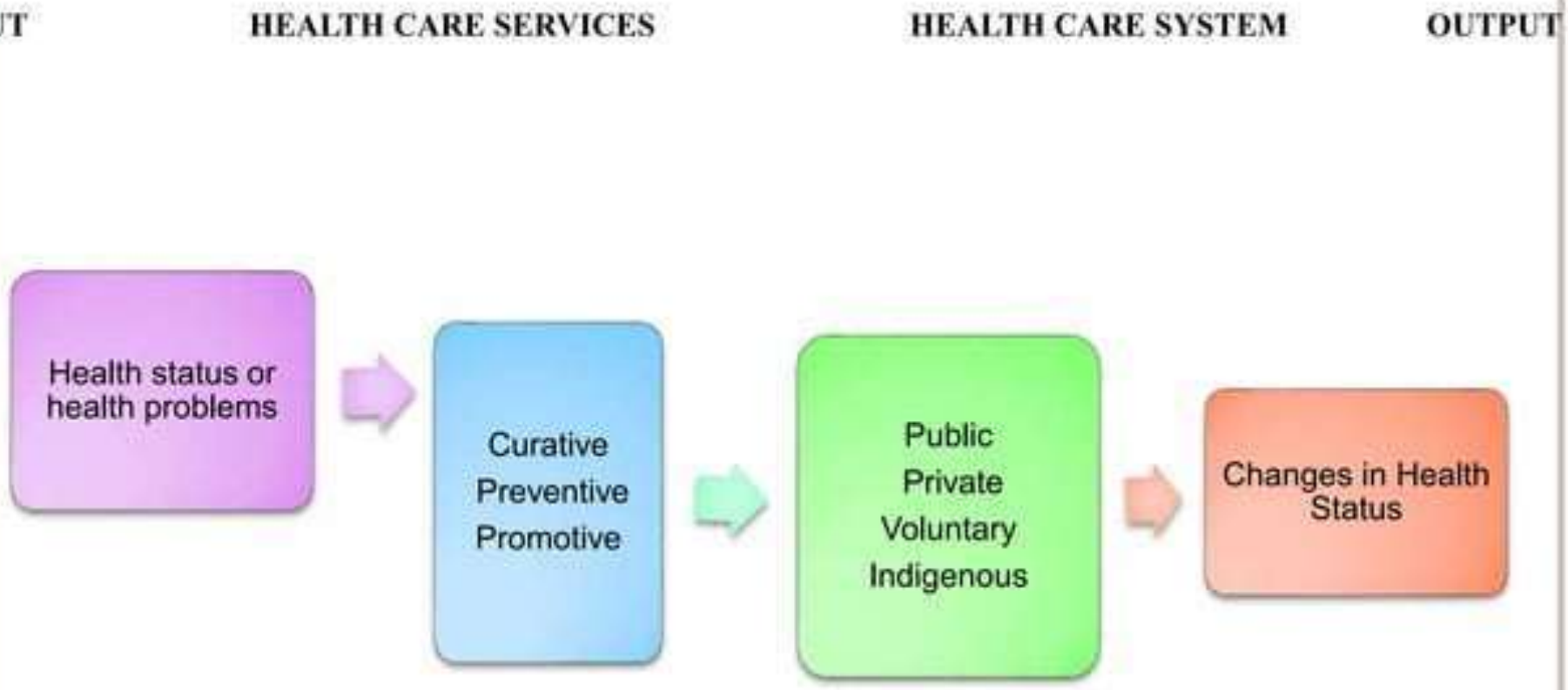
- Consist of 15-30 elected members.
- Covers the population of5000 to 20000
- Chained by the president, there is a vice president and secretary.
- Responsible for overall planning and development of the village.
- The panchayath secretary has function for wide areas such as civic administration, including sanitation and public health and socio economic development of village.

The Nyaya panchayath

- It is composed of 5 members from the panchayath.
- It tries to solve the dispute between two parties/ groups/ individuals over certain matter as mutual consent.

HEALTH CARE DELIVERY SYSTEM IN INDIA

HEALTH CARE DELIVERY SYSTEM MODEL



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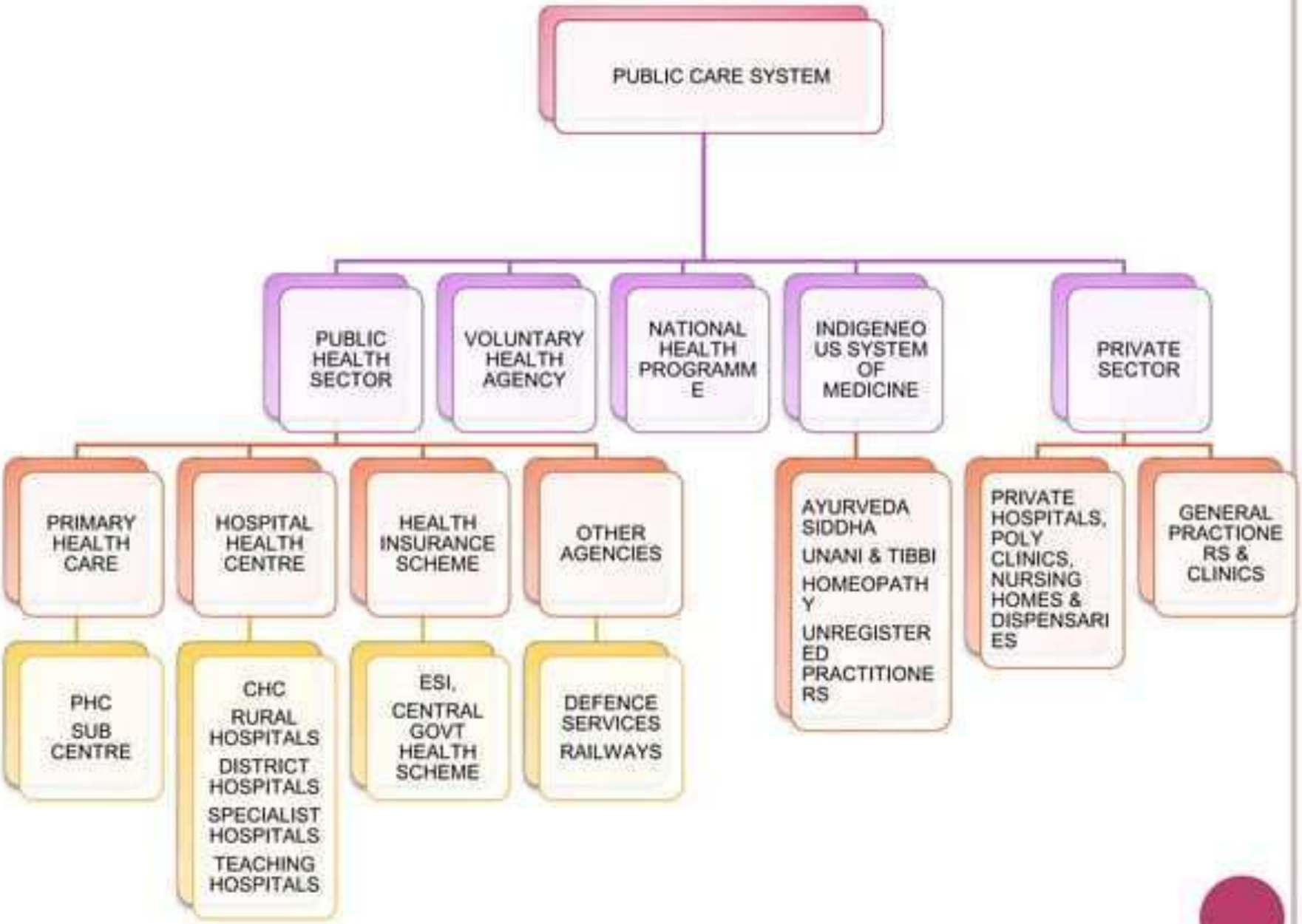
- The INPUT are the health status or health problems of the community they represent the health needs and demands of the community.
- The HEALTH CARE SERVICES are designed to meet the health needs of the community through the use of available knowledge and resources.
- The HEALTH CARE SYSTEM is intended to delivers the health care services.
- The final outcome is the changed health status or improved the health status of the community.

Health care system

In India it is represented by five major sectors or agencies which differ each other by the health technology applied and by the source of funds for operation


PUBLIC CARE SYSTEM
PUBLIC HEALTH SECTOR
VOLUNTARY HEALTH AGENCY
NATIONAL HEALTH PROGRAMM E
INDIGENEO US SYSTEM OF MEDICINE
PRIVATE SECTOR
PRIMARY HEALTH CARE
HOSPITAL HEALTH CENTRE
HEALTH INSURANCE SCHEME
OTHER AGENCIES
AYURVEDA SIDDHA
UNANI & TIBBI
HOMEOPATH Y
UNREGISTER ED PRACTITIONE RS
PRIVATE HOSPITALS, POLY CLINICS, NURSING HOMES & DISPENSARI ES
GENERAL PRACTIONE RS & CLINICS
PHC
SUB CENTRE
CHC
RURAL HOSPITALS
DISTRICT HOSPITALS
SPECIALIST HOSPITALS
TEACHING HOSPITALS
ESI,
CENTRAL GOVT HEALTH SCHEME
DEFENCE SERVICES
RAILWAYS

PUBLIC SECTOR

- Public system is government sponsored system.
- It is funded by the public funds which are generated through general taxes, the services are rendered to the people at large in rural and urban areas by three tier system developed in the-block, district and state level.
- Health services in the public sector are conducted by union or state government.

Urban health services

- This scheme includes the hospitals and institutions providing health care in urban areas.
- In order to reduce the pressure of patients in DH and Medical college hospitals, satellite hospitals are established in some places.
- There is a proposal to convert big dispensaries and hospitals working at the sub divisional level into sub divisional health centre.

CONT..

- **Urban family welfare centres**

  These centres are functioning in urban areas from 1950 to provide family planning services for urban population.
- **Urban health post**

  They are categorized in 4: A,B,C,D. These posts provide RCH, first aid, distribution of contraceptives and other services in urban areas.
- **Specialty hospitals**

  In these hospitals, only certain diseases, age group or patients with specific problems are treated and specialists and specially trained nurses care for the patients. Eg: TB hospitals, Women's hospital.
- **Teaching hospitals**

  A hospital associated with medical college comes under this category. Along with teaching and training of doctors and nurses, they provide care to the people of that area.
- **Super specialty hospitals or institutes**

  Super specialists of different system/organ or diseases are trained and patients are also treated. Eg: AIIMS .These institutes are centre of excellence for health services.

- **Other Central health services /health insurance**
- The central health service was restructured in 1982 to provide medical manpower to various units like DGHS, CGHS, GNCT etc.
- Health insurance scheme
- The MOHFW has set up a task force to explore new health financing mechanism. GOI will provide support to state govt. for health insurance scheme under NRHM.
- Employee's state insurance scheme
- ESI scheme was started in 1948.This programme provide health care to industrial labour and their families. This gives safety at the time of delivery, disease, accidents etc.
- Family planning insurance scheme
- GOI has launched this scheme wef 29/11/2005 for acceptors of sterilization and indemnity insurance cover for doctors performing sterilization both in govt. and private accredited /NGO/ corporate health facilities. MOHFW has implemented Family welfare linked health insurance scheme since 1981 to compensate the acceptors of sterilization for the loss of wages for the day on which he/ she attended the medical facility for undergoing sterilization.

Central Govt. Health Scheme(CGHS)

- CGHS was started in1954.
- Objectives:
- ✓ Give extensive medical facilities to central govt. employees and their family members.
- ✓ Save govt. from heavy expenses on medical ref

Beneficiaries:

- Central Govt. Employees and their family members
- Members of parliament
- Judges of supreme court and high court
- Freedom fighters
- Pensioners of Govt. And semi Govt. Organization
- Journalists
- Governors
- Ex vice president

Facilities

- Outdoor treatment facilities,
- Emergency services in Allopathy system
- Free medicines,
- Facilities for lab services and radiological investigations,
- Specialist consultation facilities,
- Family welfare services

OTHER GOVT. HEALTH SERVICES

- Defense medical services
- Railway medical services
- Autonomous institutes
- National Health Programmes

Cont…

- **Defense medical services**

For defense services, there are separate hospitals and health service system which provides medical care to military personnel and their family members. Defense health services come under armed forces medical services (AFMS). They are responsible for providing all preventive, curative and promotional health services.

- **Railway medical services**

Indian Railways is the biggest govt. organization with highest no. of railway employee in the world. Railway provides wide range of health services to its employees through railway hospitals, clinics and health units.

- **Autonomous institutes**

Under this category all such institution are included which receive central Govt. aid but except few important matter all other decisions are made by the institutes itself. Example AIIMS Delhi, NIMHANS Bangalore.

Cont..

National Health Programmes

- Vector born disease control programme
- National anti malaria programme
- National filarial control programme
- Kala Azar control Programme
- National leprosy eradication programme
- National iodine deficiency disorder control programme
- National programme for control programme
- National mental health programme
- National tuberculosis control programme
- National AIDS control programme
- National cancer control programme
- National G worm eradication programme
- YAWS eradication Programme
- National surveillance programme for communicable disease
- National family welfare programme

AYUSH

AYUSH is an acronym that used to refer the non-allopathi medical system in India.

It includes the Indian medical system of

- Ayurvecla,
- Yoga,
- Unani
- Siddha and
- Homeopathy

In the current terminology of the ministry of India, non allopathi doctors are now referred to as AYUSH doctors.

Cont…

- Department of India System of medicine and Homeopathy was created in March 1955
- Renamed as department of Ayurveda, Yoga and Naturopathy, Unani, Siddha and Homeopathy(AYUSH) in November 2003 with a view to providing focused attention to development of education and research in Ayurveda, Yoga and Naturopathy, Unani, Siddha and homeopathy systems.

OBJECTIVES

- To upgrade the educational standard in the Indian system of medicine and homeopathy college in the country.
- To draw up schemes for promotion, cultivation and regeneration of medicinal plants used in these systems.
- To evolve pharmacopoeial standard for Indian system of medicine and homeopathy drugs.

- **Ayurveda system of Medicine**

Ayurveda is an ancient system of medicine practiced in India. Its documentation dated back to veda period. The word Ayurveda implies the science of life. Ayurveda was done by charakain Charaka samhitha and by sushruta in sushruta samhitha.

- **Siddha system of medicine**

Siddha is one of the oldest systems of medicine in India. The team siddha implies achievement.

Basic philosophy

It believes that all objects in the universe include human body are composed of fine elements - earth, water, fire, air and space.

Unani system of medicine

Unani system of medicine has its origin in grace before Christ. It was introduced in India around the 11th century.

Theoretical basis

- The unani system of medicine not only therapeutic in nature but also deals with health promotion and presentation of disease.
- The system is based on humor theory of Hippocrat's is blood, phlegm, yellow bile and black bile. Any change or disturbances in hormones brings about change in temperature of a person attending his health status.

Homeopathy system of medicine

Homeopathy has been in practice for 170 years by thousand of practitioners and there are over 100 homeopathy, journals and worldwide.

Basic laws, diagnosis and treatment Method

- The law of direction of cure

The curative process the symptom disappear in the reverse order of it appearance from above down wards, from center to periphery

- The law of single remedy

Homeopathy generally is only a single medicine which has time simplicity of symptoms with that of remedy

- The law of minimum doses

The doses applied are the minimum possible just sufficient to correct the diseased state

- The theory of chronic disease

The concept is that physical mental and spiritual experience of sick form the totality of the disease

- **Naturopathy and Yoga**

Naturopathy

- It is base as the law of nature. It is closely associated with Ayurveda as far as it fundamental principles are concerned. It has regular attention of eating and living habits, adaption of purification measures, use of hydrotherapy, cold packs, mudpacks, baths, massage and a variety of methods.

- Yoga

Yoga is a science which helps to coordinate body and mind more effectively. It promotes mental physical, social and spiritual health.

RURAL HEALTH SERVICES

The rural health services are being implemented through a network of primary health care system: three tier system.

- **Three Tier System**

  In primary health care rural health services are developed into a three tier system according to the size of population.

Name of health centre	POPULATION STANDARD	
	PLAINS	HILLS/ TRIBAL
CHC	5000	3000
PHC	30000	20000
Sub-centre	1,20,000	80000

Sub Centre

- It is the first peripheral institution between community and health services in rural areas.It is looked after by female MPW/ANM and a male MPW.

Indian Public Health Standard for sub centre

- **Maternal health care**

Antenatal care

- Early registration of pregnancy
- Folic acid and iron supplementation
- Tetanus texoid immunization
- Identification of high rise pregnancy
- Counseling on health, diet.

Intranatal care

- Promotion of intranatal deliveries
- Skilled attendance home delivery
- Appropriate and prompt referral service

Postnatal care

- minimum 2 post natal visits
- initiation of BF within 1/2 hours
- counseling diet, hygiene and contraception
- Provision of facilities janani suraksha yojana

Child health care

- Essential new born care
- Promotion of exclusive breast feeding
- Immunization
- Vitamin A prophylaxis
- Prevention and control of childhood disease

- **Family planning and contraception**
- Education, counseling and motivation to do appropriate family planning method
- Provision of contraceptive such condom, oral pill, emergency contraceptives
- follow up services

CONT..

- Counseling and appropriate referral for safe abortion
- Adolescent health care
- Assistance to school health service
- Water quality monitoring
- Promotion of sanitation
- Field visits by appropriate health workers for disease surveillance
- Community need assessment
- Curative service for minor elements
- Training of traditional birth attendants and ASHA
- Co ordinate service of anganawadi workers
- National health Programmes

PHC

- This is the first contact point between the community and the medical officer.
- The work of PHC is looked after by medical officer in charge. There are other doctors and health workers

Indian public health standards for PHC

- The IPHS for primary health centres has been prepared keeping in view the resource available with respect to functional requirement for PHCs with minimum standards such as building, man power, instruments and equipments, drugs and other facilities etc.
- The standard prescribed are for 20,000 - 30,000 population with six beds, as all the block level PHC's are ultimately going to be upgraded as CHC with 30 beds of providing specialized services

The objectives of IPHS for PHC

- To provide comprehensive primary health care to the community through the primary health centres.
- To achieve and maintain an acceptable standard of primary health care.
- To make the service are same responsive and sensitive to the needs of the community.
- Minimum requirements at PHC for meeting the IPHs the assured services covers all the essential elements of preventive, promotive, curative and rehabilitative primary health care.

Services

- Medical care
- OPD service
- 24 Hours emergency services
- Referral services
- Inpatient services

Maternal and child health care

Maternal care

- Early registration and 3 antenatal checkups.
- Minimum laboratory investigations.
- Nutrition and health counseling
- Supplementation of folic acid and iron tablet
- Identification of high risk pregnancy
- Referral to first referral mist

Cont..

- Intranatal care
- 24 how service for normal delivery
- Promotion of institutional delivery
- Conduction of assisted delivery including vaccum and forceps
- Manual removal of placenta
- Appropriate, and prompt referral service if needed
- Postnatal care
- Minimum of 2 postpartum home visit within 42 hours of delivery and second within seven days.
- Initiation of breast feeding within 1/2 hours of delivery.
- Education on nutrition, hygiene and contraception
- provision of facilities under Janani Suraksha yojana

Cont..

- New born care
- Essential new born care
- Facilities and care for neonatal representation
- Management of neonatal hypothermia and jaundice care of the child
- Emergency care of the sick child
- Care of the routine childhood illness
- Promotion of breast feeding for 6 months
- Immunization
- Vitamin A Prophylaxis

Cont…

- **Full range of family planning service including counseling and appropriate referral for couples having infertility**
- MTP
- **Health education of RTI/STI**
- **Nutrition services**
- **School health services**
- **Adolescent healthcare**
- **Disease surveillance and central of epidemics**
- **Collection and reporting of vital events**
- **Promotion of sanitation**
- **Testing of water quality and disinfection of water sources**
- **National health Programmes**
- **Appropriate or prompt referral if needed**
- **Record of vital events:** Reporting of birth and death, maintenance of all relevant records.

Cont..

Training

- Health workers and TBA
- Initial and periodic training of paramedical in treatment of minor ailments.
- Training of ASHA
- Periodic training of duties through continuing medical education, conferences, skill development training etc
- Training of ANM, and LHV in antenatal care and skilled birth attendance.
- Training under integrated management of neonatal and childhood illness (IMNCI)
- Training of pharmacist on AYUSH
- Training of AYUSH doctor in imparting health services related to National health and family welfare programme

Basic laboratory services

- Routine urine, stool a blood test
- BT, CT
- Diagnosis of STDs with wet monitoring, gram stain etc
- Sputum testing for TB
- Blood sugar examination for malarial parasite
- Rapid tests for Pregnancy
- Test for syphilis YAWS
- Rapid diagnostic test for typhoid and malaria
- Rapid test kit for fecal contamination of water
- Estimation of chlorine level of water using orthotoluedine reagent.

Monitoring and supervision

- Maintaining and supervision of, activities of sub-centers through regular meeting
- Monitoring of all national health programmes
- Maintaining activities of ASHA
- Medical officers should visit all sub centres at least once in a month
- Health assistant male and LHV should visit sub centres once a week

Selected surgical Procedures

Main streaming of AYUSH

Record of vital events and procedure

CHC

- This works as referral centre for PHC of the area.Along with this maternity care and speciality care is also provided.Usually 4 PHCs are connected with a CHC.

Indian Public Health standards for community health centre,

In order to provide quality of health care in community, IPHs are being prescribed to provide expert care to the community and achieve and maintain an acceptable standard of quality of health care.

- Every CHC should provide following services
- Care of routine and emergency case in surgery'
- Care of routine and emergency case in medicine'
- 24 hrs delivery services including normal and assisted deliveries.
- External and emergency obstetric care
- Full range of family planning service
- Safe abortion services.

Cont…

- Newborn care
- Routine and emergency care of sick children
- Other management including nasal packing, tracheostomy, foreign body removal etc.
- A1l the National Health programme should be delivered through CHC's.

Others

- Blood storage facility
- Essential laboratory services
- Referral services

OTHER RURAL SERVICES

a).Village health guide

b). Local Dais

c. Anganwadi worker

d) . ASHA Scheme

Village health guide

- A village health guide is a person with an aptitude for social service and is not a full time government functionary.
- The village health guide scheme was introduced on 2nd october1977, with the idea of securing people participation in the case of their own health.
- The health guide is now mostly women. The health guides came from and are chosen by the community they work.
- They serve as link between the community and the governmental infrastructure.
- They provide first contact between the individual and the health system.

The guidelines for their selection are:

- They should be permanent residents of the local community, preferably women.
- They should be able to read and write, having minimum formal education at least up to VI std
- They should be acceptable to all sections of the community
- They should be able to spare at least 2 to 3 hrs every day for community health work.

CONT…

- After selection, the health guide undergo a short training in primary healthcare.
- The training is arranged in the nearest PHC, sub-centre or any other suitable place for the duration of 3 months.
- During the training period they receive a stipend of Rs 200/ month

The duties assigned to the VHG

- Treatment of simple ailments
- Activities in first aid
- Mother and child health including
- Family planning, health education and sanitation.
- They are expected to do commonly health work in their spare time of about 2 to 3 hrs daily for which they are paid an honorarium of Rs 5o/month and drugs worth Rs 600/annum.

Local Dais

- Most deliveries in rural areas are still handled by untrained daies who are often the only people immediately available to women during the prenatal period.
- The training for dais is for 30 working days. Each dai is paid a stipend Rs 300 during her training period. T
- raining is given at the PHC/ sub centre or MCH centre for 2 days in a week and on the remaining 4 days on the week, they accompany the health worker to the villages.
- During her training each dai is require to conduct at least 2 deliveries under the guidance and super vision of the HW (f), ANM or HA (f).
- After successful completion of training, each dai is provided with delivery kit and a certificate.
- The dai is also expertise to play a vital role in propagating small family norm since they are more acceptable to the community.

Anganwadi worker

- Under ICDS scheme there is an anganwadi worker for a population of 1000.
- The anganwadi worker is selected from the community she is expected to serve.
- She undergoes training in various aspects of health, nutrition and child development for 4 months.
- She is paid an honorarium Rs 1500/m for the services rendered.

Services

- Health check up
- Immunization
- Supplementary nutrition
- Health education
- Non formal preschool education
- Referral services.

ASHA Scheme

- One ASHA for 1000 population
- Chosen by and accountable to panchayath
- act as interface between the community and the public health system.

VOLUNTARY HEALTH AGENCIES

There are varieties of nongovernmental organization which are voluntary in nature and contribute tremendously in furnishing the public health by providing health services or health education by advance research etc.

Functions

- Supplement the work of government agencies
- Pioneering
- Education

Voluntary health agencies

- Indian red cross society
- Hind Kusht Nivaran Sangh
- Indian Council for child welfare
- Tuberculosis association of India
- Bharat sevak samaj
- Central social welfare board
- Kasturba memorial fund
- Family planning association of India
- All India women's conference
- The all India Blind relief society
- Professional bodies
- International agencies

PRIVATE SECTOR

- Services provided by the private sector can be classified into 3:

1. Private hospitals and clinics
2. Private consultation centres
3. Mission or religious hospital

CONT..

- **Private hospitals and clinics:** Because of mixed economy and globalization, private hospitals are being opened rapidly. Private hospitals, nursing homes and clinics are mainly therapeutic institutions and provide health services to urban population only. To get their services, price is to be paid hence poor and weaker section cannot get their services.
- **Private consultation centres :** With the increasing no. of allopathic hospitals a tendency to start health consultation privately is increasing among doctors. Medical council of India and IMA keep professional checks over them. In the rural areas fake doctors or quacks often starts private clinics and consultation which is a mockery of health care of common people and is to be checked.
- **Mission or religious hospitals:** These hospitals are managed by mission, trust or charitable institutions. In many parts of the nation, such hospitals and clinics provide medical services either free of cost or at very cheap rate to common people.

PROBLEMS AND CHALLENGES FACED BY HEALTH CARE DELIVERY SYSTEM IN INDIA

- Low public health expenditure
- Vanishing public sector
- Rural-urban disparities in the availability of health services
- Factors limiting the ability of health sector:
- Existence of small number of public health institutions than their actual requirement
- Lack of some basic facilities in govt. health institutions like electricity, water etc
- Manpower shortage in the health system
- Absentee doctors
- Inconvenient opening times
- Informal payments
- Poor condition of health infrastructure
- Limited doctor salaries
- Lack of medicines

ROLE OF NURSE IN DELIVERING HEALTH CARE

Assessment

- Community survey
- Undertake a comprehensive health survey of the population sample
- Examine existing health care delivery in the area
- Obtain relevant statistical data from local health facilities
- Assess local health needs and their perception.
- Assess local health demands and their relevance
- Assess local health resources and their mobilization potential

Community diagnosis

- Information collected through community survey is analysed to identify problems for ill health of community
- The problems identified in our nation are:
- Health problems like communicable diseases, lack of safe water, insanitation, unhygienic housing, uncontrolled vector breeding, unhealthy habits and practices.
- Indirectly related health problems like poverty, illiteracy, ignorance, unemployment, underemployment, uncontrolled reproduction.
- Service related problems like poor health care facilities, untrained primary health care staff, inadequate drugs and supplies, inadequate monitoring and supervision.

Planning

- Setting of objectives: Objective setting depends on nature of problem, resources available and the means of intervention envisaged. Objective should be clear , quantifiable and feasible
-
- Formulation of strategies: Strategy is an approach or an action plan for achievement of set objectives. Several alternative approaches are evolved. Each of these approaches is subjected to critical analysis and the most appropriate one is selected for evolvement of programme strategy.
- Mobilization of resources: Keeping in mind the objectives and strategies, appropriate resources are generated to sustain the programme.

Implementation

- Implementation of programme starts with the establishment of an appropriate health infrastructure capable of achieving the set objectives by applying the formulated strategy
- Training and orientation of man power and provision for their supportive supervision
- Need sustained administrative and logistic support to ensure continuous supply of materials and requirements.

Evaluation

- Systemic process of assessment of progress and achievement of a programme. The ultimate achievements of a programme are assessed by terminal evaluation in terms of
- Programme efficiency :measure of justifying the investment of resources
- Programme effectiveness : measure of success of programme in achievement of set objectives
- Programme impact: measure of overall effects of programme on health and development.

THANK YOU